



Gavin Newsom, Governor
State of California
Health and Human Services Agency
DEPARTMENT OF MANAGED HEALTH CARE
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January 22, 2026

VIA ELECTRONIC MAIL

Lisette Rendon
California Physicians' Service
4201 Town Center Drive
El Dorado Hills, CA 95762

Re: Proposed Contract Termination with Providence St. Joseph Hospital
Block Transfer Filing No. 2026-HOSP-189
Filed on January 15, 2026

Dear Ms. Lisette Rendon:

The Department of Managed Health Care (Department) is reviewing the information in the above-referenced Block Transfer Filing (Filing) submitted by **California Physicians' Service** (Plan) for compliance with the requirements of the Knox-Keene Health Care Service Plan Act of 1975, as amended (Cal. Health & Safety Code § 1340 *et seq.*) and the regulations promulgated thereunder (Cal. Code Regs., Tit. 28, § 1000 *et seq.*). In the event of contract termination, the Filing proposes to remove **Providence St. Joseph Hospital** (Terminating Hospital) from the Plan's hospital network. The effective date of contract termination between the Terminating Hospital and the Plan (Termination Date) is currently **April 30, 2026**.

Please review the following comments and feel free to contact me if you would like to discuss any of the issues before filing the Plan's response.

1. Regarding the Enrollee Transfer Notice submitted for this Filing, the following information was found missing. Please resubmit the Enrollee Transfer Notice for the Department's review to include the following:
 - a. Please include the names of all Alternate Hospitals for this Block Transfer Filing.

Response: Plan advises in our approved Enrollee Transfer Notice (ETN), the language indicates a listing of some of the alternate facilities in our network. It is not prudent to list every facility in our network; therefore, we have included instructions in our ETN to

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check participating status. Hospitals don't limit services to people who live in specific ZIP Codes. Zone Improvement Plan areas do not dictate where a person receives acute care services.

2. Please confirm that the Plan is aware that it cannot transition its enrollees or send Enrollee Transfer Notices without the Department's approval.

Response: Plan confirms it is aware that it cannot transition its enrollees or send Enrollee Transfer Notices without the Department's approval

3. Please confirm whether the hospital termination is part of a health system termination. If yes, please confirm the health system and whether the contract is a stand-alone contract.

Response: Plan confirms Providence Little Company of Mary MC - Torrance contract is a stand-alone contract

4. Please confirm whether the contract termination between the Plan and Providence St. Joseph Hospital impacts Point of Service (POS) enrollees.

Response: Plan confirms the contract termination between the Plan and the terminating hospital impacts enrollees with the Point of Service (POS) benefit.

5. The Plan's PPO line of business is licensed by the Department. Please confirm the following:

- a. Whether the PPO providers that routinely utilize the Terminating Hospital have adequate admitting privileges for PCPs and specialists to at least one of the identified Alternate Hospitals containing all services that will be lost should the contract ultimately terminate,

Response: Plan confirms Plan will send an Enrollee Transfer Notice approved by the Department to all PPO enrollees with a 15-mile radius of the Terminating Hospital.

- b. The Plan will send an Enrollee Transfer Notice approved by the Department to all PPO enrollees with a 15 mile radius of the Terminating Hospital.

Response: Plan confirms the PPO providers can admit for and provide all medically necessary services at the Alternate Hospitals

6. For each affected provider group that primarily utilizes the Terminating Hospital, please summarize or confirm the following:

- a. The ability of the provider group to redirect enrollees to the Alternate Hospitals:

- i. Whether there is any specialty for which all of the associated specialists have admitting privileges **only** at the Terminating Hospital and not at any of the Alternate Hospitals,
 1. If yes, please identify the specialty and explain how Plan enrollees assigned to the provider group will receive care and services for that specialty following contract termination.
- ii. If the provider group does not have any primary care physicians or specialists with admitting privileges to one or more Alternate Hospitals, please explain how its assigned enrollees will be admitted to the Alternate Hospital(s) (e.g. via hospitalists, Plan admission process, etc.).
 1. If hospitalists are utilized, please indicate:
 - a. The name of the group,
 - b. The number of associated hospitalists, and
 - c. If the hospitalists can admit enrollees for any medically necessary services.

Response: Plan asserts that any admitting HMO medical groups will be advised to redirect admissions to participating facilities. If a delegated medical group authorizes an admission or service to the Terminating Hospital after the contract termination date, the Plan will process the institutional claim according to the enrollees' Department approved Evidence of Coverage. The Plan allows out of network referrals when a covered service is not available, accessible, or can be provided timely at a participating network hospital or specialty facility within the Plan's network and will allow members to receive that service at a non-participating hospital, as appropriate. The member's policy's in-network benefit levels are applied to authorized out of network referrals. The Plan confirms that Plan enrollees, including POS enrollees using their HMO benefits, may access the Terminating Hospital for authorized covered services at in-network benefit levels.

7. For each Alternate Hospital, please summarize or confirm the following:
 - a. The Plan contacted the hospital regarding the Block Transfer,
Response: Blue Shield of California contacts the alternate hospitals identified for each Block Transfer Filing. Contact with the hospitals' representatives is prioritized on occupancy rates and service inventories that are required to meet the needs of our members who could be potentially redirected to one of the alternate facilities. The Plan verifies that the alternate hospitals have sufficient available bed capacity to accept additional enrollee admissions should the contract with the terminating hospital actually terminate. Occasionally, when an alternate hospital fails or refuses to respond to a capacity request, it is necessary to refer to the hospital's website and/or hospital self-reported information to the state.

- b. Any concerns expressed by the hospital regarding its ability to accommodate the hospital-based needs of enrollees redirected by the Block Transfer,

Response: The alternate hospitals did not express concerns regarding their ability to accommodate the hospital-based needs of enrollees who may be redirected as a result of the potential contract termination between the Plan and the terminating hospital.

- c. In the case of an evergreen contract with any proposed Alternate Hospital, please provide the Department with:
 - i. Contract renewal date(s), or
 - ii. If there is no specified renewal date(s), please state whether a notice of termination has been sent or received, and
 - iii. Confirm whether each evergreen contract will automatically renew.

- 8. Please confirm whether the HMO providers that routinely utilize the Terminating Hospital have adequate admitting privileges for PCPs and specialists to at least one of the identified Alternate Hospitals containing all services that will be lost should the contract ultimately terminate.

Response: Plan confirms the HMO providers that routinely utilize the Terminating Hospital have adequate admitting privileges for PCPs and specialists to at least one of the identified Alternate Hospitals containing all services that will be lost should the contract ultimately terminate.

- 9. The Plan submitted University of California Irvine Medical Center as an Alternate Hospital. According to the California Department of Health Care Access and Information (formerly OSHPD), this Alternate Hospital has a licensed bed occupancy rate greater than 80 percent. Please confirm the following:

- a. If the Plan has contacted the hospital and whether the hospital has expressed concerns regarding the sufficiency to serve additional Plan enrollees,
- b. The Plan's protocol to ensure enrollees maintain access to all medically necessary services, in the event the hospital does not have capacity to serve additional Plan enrollees, and
- c. Please provide the Department with the Plan's methodology for determining no anticipated capacity issues.

Response: Plan uses 80 percent as the threshold for normal facility occupancy. When available bed occupancy rates exceed 80 percent, Blue Shield's practice is to speak with representatives from the hospital to ascertain if there are any capacity issues or concerns as well as identify if the hospital has efforts to staff for potentially higher occupancy. In most cases, hospital representatives do not have concerns related to capacity concerns and are able to expand occupancy to full licensed bed capacity if necessary. Blue Shield assures its members that they will have timely

access to care. If a service is not available at an alternate participating hospital due to capacity, scheduling delays, distance, or a lack of physician admitting privileges, then out-of-network referrals will be reviewed on a case by case basis. When an out-of-network referral is approved, the member's in-network benefit levels are applied to associated claims

10. In addition to the in-patient services comparison on the Block Transfer portal, please indicate any of the following services that are available at the Terminating Hospital, but not available at the Alternate Hospitals:

- a. Any out-patient or in-patient services

If any services are not available at the Alternate Hospitals, please explain where affected enrollees will receive these services following contract termination.

Response: The Terminating Hospital will be a non-participating facility after the contract termination date. Any admitting HMO medical groups will be advised to redirect admissions to participating facilities. If a delegated medical group authorizes an admission to the Terminating Hospital after the contract termination date, the Plan will process the institutional claim according to the enrollees' Department approved Evidence of Coverage. The Plan allows out of network referrals when a covered service is not available, accessible, or can be provided timely at a participating network hospital or specialty facility within the Plan's network and will allow members to receive that service at a non-participating hospital, as appropriate. The member's policy's in-network benefit levels are applied to authorized out of network referrals. The Plan confirms that Plan enrollees, including POS enrollees using their HMO benefits, may access the Terminating Hospital for authorized covered services at in-network benefit levels.

11. On the Block Transfer Filing form, for the provider groups listed below, the Plan reports the same number of Specialists with Active Admit Privileges at Terminating Hospital as the number of Specialists with Active Admit Privileges at Alternate Hospital. Please provide a justification as to why the number of specialists reported is the same. Additionally, please confirm how the Plan ensures there are an adequate number of specialists that can admit to the Alternate Hospital(s).

- a. Torrance Health Association, Inc. (DBA: Torrance Memorial IPA) (RBO)
- b. St. Jude Heritage Medical Group (RBO)
- c. High Desert Primary Care Medical Group (RBO)
- d. St. Joseph Hospital Affiliated Physicians (RBO)
- e. PrimeCare Medical Group of Corona, Inc. (RBO)
- f. Monarch Healthcare, A Medical Group, Inc. (RBO)

- g. St. Joseph Heritage Medical Group (RBO)
- h. Hoag Clinic (RBO)
- i. Greater Newport Physicians Medical Group (RBO)
- j. Altamed Health Services Corporation (RBO)

Response: Blue Shield's members are not assigned to hospitals and therefore are not "block transferred" to another facility when the contract terminates. Rather, members may obtain services from any participating hospital to which their physician or medical group maintains admitting privileges or otherwise arranges for admission (for example, use of specialty referral, a hospitalist, or Hospitalist Program). The admitting medical groups' physicians have equal means to admit to all the hospitals used by their affiliated physicians, which are listed in the medical groups' contracts with the health plan. For these reasons, the numbers of admitting physicians are the same for each of the hospitals used by the medical groups' affiliated physicians. The physicians admit patients to the hospitals used by their medical group through direct admitting privileges, physician referrals, temporary admitting allowances, hospitalists, or a contracted hospitalist program. Please note, if/when a physician cannot admit a patient to an alternate facility for a needed covered service/procedure due to a lack of hospital privileges, capacity issues, care delays that can harm the patient, distance where the health plan does not have alternative access standards to acute care, etc. the admission to the terminating facility is authorized at in-network benefit levels.

12. Please confirm the total number of bed days, admits, and enrollees assigned to each provider group listed on the table below that primarily utilize the Terminating Hospital:

Response: The plan has attached a corrected grid below.

IPA Name	Bed Days	Admits	Enrollees Assigned to Provider Group
Torrance Health Association, Inc. (RBO)	3	1	10,363
St. Jude Heritage Medical Group (RBO)	5	1	19,651
High Desert Primary Care Medical Group (RBO)	21	14	7,970
St. Joseph Hospital Affiliated Physicians (RBO)	19	6	13,044
PrimeCare Medical Group of Corona, Inc. (RBO)	3	1	2,478
Monarch Healthcare, A Medical Group, Inc. (RBO)	2	1	6,872
St. Joseph Heritage Medical Group (RBO)	4	2	11,935
Hoag Clinic (RBO)	4	2	4,643

Greater Newport Physicians Medical Group (RBO)	3	1	7,788
Altamed Health Services Corporation (RBO)	8	2	2,129
Grand Total	72	31	86,873

- a. On the Block Transfer Filing Portal, the Plan indicated a total of 75,862 enrollees who will be redirected. However, on the Block Transfer Filing Form, there are ten provider groups that routinely utilize the Terminating Hospital with total of 86,873 impacted enrollees. Please reconcile the discrepancy.
Response: Plan confirm the total number of enrollees assigned to the medical groups is 86,873.
- b. On the Block Transfer Filing Portal, the Plan indicated a total of 72 bed days and 31 admits. However, on the Block Transfer Filing Form, the total is 70 bed days and 30 admits. Please reconcile the discrepancy.
Response: Plan confirms the total number of bed days is 72 and 31 admits
12. Please confirm the medical groups that do not utilize the Terminating Hospital as a primary hospital, but have admitted enrollees for the most recently completed calendar year, are able to refer their enrollees to other hospitals within the Plan's network for services comparable to those provided by the Terminating Hospital.
Response: Plan confirms the medical groups that do not utilize the Terminating Hospital as a primary hospital, but have admitted enrollees for the most recently completed calendar year, are able to refer their enrollees to other hospitals within the Plan's network for services comparable to those provided by the Terminating Hospital.
13. Please identify the number of enrollees who have a procedure scheduled and authorized by the Plan at the Terminating Hospital post-termination.
Response: Plan asserts at this time there are no enrollees who have a procedure scheduled and authorized by the Plan at the Terminating Hospital post-termination.
14. The Plan submitted filing, 2026-PROV-78, for the potential termination of High Desert Primary Care Medical Group (RBO). In addition, its Receiving Provider Groups contained the following hospitals as primarily utilized hospitals:

ARROWHEAD REGIONAL MEDICAL CENTER	00 N. PEPPER AVENUE, COLTON, 92324
DESERT REGIONAL MEDICAL CENTER	150 NORTH INDIAN CANYON DRIVE, PALM SPRINGS, 92262
DESERT VALLEY HOSPITAL	6850 BEAR VALLEY ROAD, VICTORVILLE, 92395
EMANATE HEALTH QUEEN OF THE VALLEY HOSPITAL	115 SOUTH SUNSET AVENUE, WEST COVINA, 91790
LOMA LINDA UNIV. MED. CENTER EAST CAMPUS HOSPITAL	5333 BARTON ROAD, LOMA LINDA, 92354
LOMA LINDA UNIVERSITY CHILDREN'S HOSPITAL	1234 ANDERSON ST., STE. A, LOMA LINDA, 92354
LOMA LINDA UNIVERSITY MEDICAL CENTER	1234 ANDERSON STREET, LOMA LINDA, 92354
LOMA LINDA UNIVERSITY MEDICAL CENTER-MURRIETA	8062 BAXTER ROAD, MURRIETA, 92563
PROVIDENCE SAINT JOSEPH MEDICAL CENTER	501 S BUENA VISTA STREET, BURBANK, 91505
PROVIDENCE ST. JOSEPH HOSPITAL	100 WEST STEWART DRIVE, ORANGE, 92868
PROVIDENCE ST. JUDE MEDICAL CENTER	01 EAST VALENCIA MESA DRIVE, FULLERTON, 92835
PROVIDENCE ST. MARY MEDICAL CENTER	8300 US HIGHWAY 18, APPLE VALLEY, 92307
RONALD REAGAN UCLA MEDICAL CENTER	757 WESTWOOD PLAZA, LOS ANGELES, 90095
UNIVERSITY OF CALIFORNIA IRVINE MEDICAL CENTER	01 CITY DRIVE SOUTH, ORANGE, 92868

However, 2026-HOSP-189 lists the following Alternate Hospitals and does not list all primarily utilized hospitals for enrollees that may be ultimately transferred to Receiving Groups listed in 2026-PROV-78:

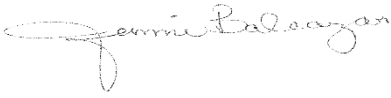
- AHMC Anaheim Regional Medical Center
 - Anaheim Global Medical Center
 - Chapman Global Medical Center
 - Foothill Regional Medical Center
 - Healthbridge Children's Hospital – Orange
 - University of California Irvine Medical Center
- a. Please explain why the hospitals listed as primarily utilized for the Receiving Provider Groups in 2026-PROV-78 are not included as Alternate Hospitals in 2026-HOSP-189. If these hospitals are not considered alternates, please provide the justification.

Response: Plan listed alternate facilities within the 15/30 standard in 2026-HOSP-189. In 2026-PROV-78 Plan listed all admitting facilities for the receiving provider groups. Plan asserts Hospitals don't limit services to people who live in specific ZIP Codes. Zone Improvement Plan areas do not dictate where a person receives acute care services. As a member of the Blue Cross Blue Shield Association, we pay claims for members throughout the nation, and even internationally, for any participating Plan facility. Any of our members or BCBSA affiliate members can receive acute care services at a contracted hospital no matter how far away they live.

Please submit the Plan's response by **January 29, 2026**, via the Block Transfer portal as a "Response to Comment Letter."

Please contact me if there are any questions regarding the above.

Sincerely,

A handwritten signature in cursive script, reading "Jennie Balcazar". The signature is written in dark ink on a white background.

Jennie Balcazar
Analyst I
Office of Plan Monitoring